Strategic Cross-Jurisdictional Access to Sensitive Personal Data: A Legal and Operational Framework

Executive Summary

This report establishes a comprehensive operational framework for an Irish national resident in London to exercise their data protection rights across the distinct legal jurisdictions of the Republic of Ireland (EU GDPR) and Northern Ireland (UK GDPR). The primary objective is the exhaustive retrieval of complex medical records, psychiatric notes, institutional emails, and governance documentation from five specific public bodies: University of Galway, University College Hospital Galway (Saolta Group), Belfast Health and Social Care Trust, Royal Victoria Hospital Belfast, and Queen's University Belfast.

The central challenge identified is the requirement for the **digital delivery** of high-volume, sensitive datasets—including high-resolution radiological imaging and acute psychiatric notes—to a remote jurisdiction. This requirement is not merely a preference but a necessity legally underpinned by the data subject's neurodivergent status, specifically Traumatic Brain Injury (TBI) and Complex Post-Traumatic Stress Disorder (C-PTSD).

This analysis constructs a robust legal argument leveraging the **Equal Status Act 2000 (Ireland)** and the **Disability Discrimination Act 1995 (Northern Ireland)** to mandate digital formats as a "reasonable accommodation" or "reasonable adjustment." It further addresses the high probability of data controllers invoking "Serious Harm" exemptions to withhold psychiatric records, detailing a preemptive strategy utilizing a **Nominated Health Professional** to ensure access rights are facilitated rather than extinguished.

Strategic Overview of Target Data Repositories

Institution	Jurisdiction	Data Category	Primary Legal Instrument	Key Risk Factor
University of Galway	Ireland (EU)	Counselling, Health, Security, Governance Emails	GDPR Art. 15; Equal Status Act 2000	"Disproportionate Effort" refusal for email retrieval; Mental Health redaction.
UCHG (Saolta Group)	Ireland (EU)	Acute Mental Health, Radiology (NIMIS)	GDPR Art. 15; S.I. No. 121/2022	Refusal to provide digital DICOM images via portal; Section 60 restrictions.
Belfast City Trust	UK (NI)	Psychiatric (Ward	UK GDPR; DPA	"Serious Harm"

		5), Inpatient Notes	2018 Sch 3	test (Schedule 3); Physical verification barriers.
Royal Victoria Hospital	UK (NI)	Radiology (NIPACS), A&E, Consultant Notes	UK GDPR; DDA 1995	Legacy paper records vs. Digital systems; Identity verification.
Queen's University Belfast	UK (NI)	Whistleblowing (Finlay), Security, Wellbeing	UK GDPR; Public Interest Disclosure (NI) Order	Conflict between Personal Data rights and Corporate/Privileged data.

Part I: Jurisdictional Legal Frameworks and the "Digital First" Mandate

To successfully compel these institutions to digitize and transmit years of historic records, the request must transcend the standard Subject Access Request (SAR) mechanism. It must integrate disability rights legislation directly into the data protection workflow, shifting the burden of proof onto the institution to demonstrate why digital provision is impossible, rather than merely inconvenient.

1. The Right of Access Under GDPR (EU & UK)

Both the Republic of Ireland and the United Kingdom operate under the principles of the General Data Protection Regulation (GDPR). Although the UK has exited the European Union, the provisions of the GDPR have been retained in domestic law as the "UK GDPR," sitting alongside the Data Protection Act 2018 (UK).

Article 15 (Right of Access) guarantees the data subject the right to obtain confirmation as to whether or not personal data concerning them is being processed, and, where that is the case, access to the personal data.[1] This right is fundamental and extends to all "personal data," which is defined broadly to include any information relating to an identified or identifiable natural person. This encompasses objective medical data (e.g., X-rays, blood test results) and subjective data (e.g., opinions expressed in emails or clinical notes).

The Argument for Digital Format: Article 15(3)

A critical, often underutilized provision for remote access is **Article 15(3)** of the GDPR. It stipulates that:

"Where the data subject makes the request by electronic means, and unless otherwise requested by the data subject, the information shall be provided in a commonly used electronic form."

Institutions frequently default to printing emails or photocopying medical notes, citing "system limitations" or "security policies." However, if the request is submitted via email (electronic means), the **default legal position** is that the controller *must* respond electronically.
For a resident in London requesting data from Galway or Belfast, this is not just a convenience but a logistical necessity. The tactic here is to explicitly state that paper copies are **functionally inaccessible** due to TBI/C-PTSD. This explicitly links the format of the data to the usability of the right itself. If a controller provides data in a format the subject cannot use due to disability, they have effectively denied the right of access.

2. The Disability Mandate: Reasonable Accommodation & Adjustments

The most powerful lever in this strategy is the intersection of Data Protection and Disability Law. You are requesting a **legal modification of service delivery**, transforming the request from a standard administrative procedure into a compliance matter regarding equality and non-discrimination.

Republic of Ireland: Equal Status Act 2000-2018

The **Equal Status Act 2000** prohibits discrimination in the provision of goods and services—including services provided by public bodies like universities and hospitals—on the grounds of disability.[2]
Reasonable Accommodation (Section 4): The Act places a statutory duty on service providers to do all that is "reasonable" to accommodate the needs of a person with a disability by providing special treatment or facilities. This applies if, without such special treatment, it would be "impossible or unduly difficult" for the person to avail themselves of the service.[2]
In the context of this report:

- **Service:** The provision of data under a Subject Access Request.
- **Unduly Difficult:** For a TBI survivor, the cognitive fatigue associated with processing unsorted, non-searchable physical boxes of paper constitutes an undue difficulty. The "impossible" threshold is met by the inability to effectively retrieve information from such a mass without digital assistive tools (search functions, screen readers).
- **Accommodation:** The provision of data in a digital, searchable format (e.g., OCR-enabled PDF, PST email archives) is the requisite accommodation.

Northern Ireland: Disability Discrimination Act 1995 (DDA)

It is crucial to note that Northern Ireland does not operate under the UK's Equality Act 2010. Instead, it retains the **Disability Discrimination Act 1995 (DDA)**.[5]
Duty to Make Adjustments (Sections 19-21):
Public bodies in Northern Ireland have a duty to make reasonable adjustments where a practice, policy, or procedure makes it "impossible or unreasonably difficult" for disabled persons to make use of a service.

- **Relevance to Belfast Trust/QUB:** If the Belfast Trust insists on a policy of "viewing notes in person" (a common policy for sensitive psychiatric records to ensure clinical support is available), this policy renders access "unreasonably difficult" for a

London-based requester with a disability that precludes easy travel or stressful in-person interactions.
- **The Adjustment:** The Trust *must* find an alternative, such as secure encrypted file transfer to the subject or their nominated representative. A refusal to digitize legacy paper notes for a disabled requester would likely constitute a failure to make a reasonable adjustment, as the cost of digitization is rarely high enough to be considered "unreasonable" for a large public body.[6]

3. The "Serious Harm" Exemption & The Nominated Health Professional

Both jurisdictions allow data controllers to withhold health data if disclosure would be likely to cause "serious harm" to the physical or mental health of the data subject. This is the single most significant risk to the retrieval of the **Ward 5 (Psychiatric)** notes from Belfast and the **Mental Health Unit** notes from UCHG.

Republic of Ireland: Data Protection Act 2018 (Section 60)

Under **Section 60(5)** of the Data Protection Act 2018 and the specific **Data Protection Act 2018 (Access Modification) (Health) Regulations 2022 (S.I. No. 121 of 2022)** [8]:

- A controller (e.g., UCHG) does not simply refuse access. They are mandated to consult with an "appropriate health professional" before withholding data.
- **Nominated Health Professional Mechanism:** If the decision is made to withhold data to protect the subject, the regulations allow for the data to be offered to a health professional nominated by the data subject. This professional (e.g., a GP or psychiatrist) can then review the data and decide if and how to share it with the patient.[9]

Strategic Implication: The request must *preemptively* nominate a London-based GP or Consultant. This prevents the hospital from simply closing the file or demanding the patient travel to Galway to meet a hospital consultant. It keeps the data moving towards the subject, even if it must pass through a clinical intermediary.

UK (Northern Ireland): Data Protection Act 2018 (Schedule 3, Part 2)

The UK Data Protection Act 2018, **Schedule 3, Paragraph 5**, contains the "Serious Harm Test".[10]

- Exemption applies to the extent that the application of the right of access would be likely to cause serious harm to the physical or mental health of the data subject or another individual.
- **Verification:** The controller must obtain an opinion from an "appropriate health professional" (usually the responsible clinician) unless they have one from the last six months.
- **Access via Intermediary:** While the UK legislation is less explicit about the *automatic* right to transfer to a nominee than the Irish S.I. 121/2022, the Information Commissioner's Office (ICO) guidance strongly supports this pathway as a means of facilitating access while mitigating harm.

Part II: Medical-Legal Justification (TBI & C-PTSD)

The success of the "Reasonable Accommodation" argument relies on articulating *why* the standard format is inaccessible. A generic claim of "disability" is insufficient; the report must draw a causal link between the condition and the requested format.

Traumatic Brain Injury (TBI) and Information Processing

TBI is characterized by deficits in executive function, including difficulties with organization, sequencing, and working memory.

- **Cognitive Load:** Reviewing paper files requires physical sorting, spatial organization, and the ability to hold multiple pieces of information in working memory while physically navigating a file. For a TBI survivor, this "cognitive load" can lead to rapid fatigue and an inability to comprehend the material.
- **Searchability:** Digital files allow for keyword searching (Ctrl+F), which acts as an external memory aid, bypassing the deficit in organic memory retrieval. This makes digital formats an essential assistive technology, not a luxury.

Complex PTSD (C-PTSD) and Re-Traumatization

C-PTSD involves emotional dysregulation and severe anxiety responses to trauma triggers.

- **Control and Pacing:** Digital access allows the subject to review difficult material in a controlled, safe environment (home) at their own pace, using screen-reading software if necessary to create emotional distance.
- **Avoidance of Institutional Stress:** Compelling a C-PTSD sufferer to physically attend a hospital or university office to "view" files can be re-traumatizing. Remote digital access eliminates this confrontation, serving as a reasonable accommodation to the subject's psychological needs.

Part III: Operational Analysis of Republic of Ireland Targets

Target 1: University of Galway (NUI Galway)

Scope: Counselling, Student Health, President/Dean Offices, Security.

Data Landscape & Retrieval Strategy

1. **Student Counselling & Health Unit:**
    - **Data Type:** Special Category (Health) Data. Likely stored in a dedicated practice management system (e.g., Socrates) separate from the main student record system (Banner).
    - **Risk:** Counselling services are highly protective and may claim the "Serious Harm"

exemption to protect the therapeutic process.
    - **Strategy:** Explicitly cite **S.I. No. 121 of 2022** in the opening request. State that if they believe harm is likely, the notes must be released to your London GP (provide details). This signals that you understand the law and removes their ability to simply say "no".[9]
2. **Office of the President & Dean of Students:**
    - **Data Type:** Unstructured data (emails, memos, welfare reports, disciplinary logs).
    - **Risk:** "Disproportionate Effort." Searching "all emails" for a student name can generate thousands of hits (mailing lists, newsletters).
    - **Strategy:** Refine the scope to overcome this refusal. Request: "All emails sent or received by or their direct support staff between containing the terms:,, 'Welfare', 'Complaint', 'Hospital', 'Police', 'Security'." This specific targeting makes the search "reasonable" under GDPR.
3. **Security Office:**
    - **Data Type:** Incident logs and CCTV.
    - **Retention:** CCTV is typically overwritten after 28-31 days.[11] If the events are historical, the footage is likely destroyed *unless* it was exported as part of an investigation file.
    - **Strategy:** Request "All incident reports logs referring to and any CCTV footage preserved as part of an investigation file or disciplinary process."

Contact Point: Data Protection Officer, Room A129, The Quadrangle. Email: dataprotection@nuigalway.ie.[12]

Target 2: University College Hospital Galway (Saolta Group)

Scope: Mental Health Unit, MRI, X-Ray.
Governance Note: The University of Galway and University College Hospital Galway (UCHG) are separate legal entities. UCHG is managed by the **Saolta University Health Care Group** under the **Health Service Executive (HSE)**. The SAR must be directed to the HSE/Saolta, not the University.[14]

Mental Health Unit Records

These records are often held in paper "charts" in the Medical Records Library or the Psychiatric Unit itself.

- **Digitization:** The HSE often provides photocopies. You must demand digitization (scanning to PDF) as a reasonable accommodation for TBI. The argument is that physical pages are unmanageable for your condition.
- **Serious Harm:** This is the highest-risk area for Section 60 restriction. The nomination of a health professional is mandatory here to ensure success.

Radiology (MRI / X-Ray) – The NIMIS System

Ireland utilizes the **National Integrated Medical Imaging System (NIMIS)**, a PACS (Picture Archiving and Communication System) that stores radiology imaging digitally across public

hospitals.[15]

- **Data Existence:** Your MRI and X-ray images exist as high-resolution digital **DICOM** files. They do not need to be "digitized"; they *are* digital.
- **Delivery Mechanism:** The standard HSE output is a CD/DVD.
- **London Access Issue:** Modern laptops often lack disk drives.
- **Strategy:** Request a **secure electronic transfer** (e.g., via the HSE's secure file transfer service). If they refuse due to file size (DICOM studies can be large), request the CD be mailed to London, but explicitly note that this is a "secondary preference" to electronic transfer. If you receive a CD, any local hospital or private imaging center in London can upload it to a cloud viewer for you, or you can use free DICOM viewer software (e.g., Horos, MicroDicom).

Contact Point: Freedom of Information / Data Protection Department, University Hospital Galway. Email: foi.guh@hse.ie or consumer.affairs@hse.ie (Saolta Group).[14]

Part IV: Operational Analysis of Northern Ireland Targets

Target 3: Belfast Health and Social Care Trust (BHSCT)

Scope: Belfast City Hospital (Psychiatric Ward 5), Royal Victoria Hospital (RVH).

Belfast City Hospital (Ward 5 - Psychiatric)

Ward 5 is an acute mental health setting. The notes will be extensive, handwritten (historical), and highly sensitive.

- **Serious Harm (UK DPA 2018):** The Trust will screen these notes. If they find content that could trigger a self-harm response or severe distress, they will redact or withhold it.
- **Nominated Professional:** Unlike Ireland's S.I. 121/2022, the UK does not have a statutory instrument *mandating* release to a nominee, but the ICO Code of Practice recommends it. You must frame this as a "safe harbor" option: "Should you find any data exempt under the Serious Harm test, I request you exercise your discretion to release it to my treating clinician rather than withholding it entirely."

Royal Victoria Hospital (RVH) - Radiology & A&E

- **Radiology (NIPACS):** Northern Ireland uses the **NIPACS** system (Northern Ireland Picture Archiving and Communications System). Like NIMIS, this is fully digital.
    - **Request:** "Full resolution DICOM images from NIPACS and the associated Radiologist reports."
- **A&E Notes:** These may be on the **Symphony** system (digital) or paper charts depending on the date. Request "digital exports of Symphony records and scanned copies of clinical charts."

Contact Point: Data Protection Officer, Belfast Trust, Knockbracken Healthcare Park. Email: DataProtection@belfasttrust.hscni.net.[18]

Target 4: Queen's University Belfast (QUB)

Scope: Wellbeing, Security, Alistair Finlay Whistleblower Policy Notes.

The "Alistair Finlay" Context

Alistair Finlay was a senior PSNI officer who became the Acting University Secretary at QUB.[19] The request for "whistleblower policy notes" implies a complexity: are you seeking notes *about* your whistleblowing, or notes *about the policy itself*?

1. **Personal Data (SAR):** If you blew the whistle, any report, investigation file, or email discussing your disclosure is your personal data.
    - **Request:** "All personal data held in the University Secretary's Office, specifically regarding my Protected Disclosure/Whistleblowing case."
2. **Policy Governance (FOI):** If you want to know how the policy was drafted or Finlay's role in it, this is *not* personal data. It is public administrative data.
    - **Request:** "Copies of the Whistleblowing Policy drafting minutes and governance memos authored or approved by Alistair Finlay."
3. **Hybrid Strategy:** You must submit a **Hybrid Request** to QUB. A pure SAR might miss the policy context; a pure FOI would redact your name. The template below combines both.

Contact Point: Information Compliance Unit. Email: info.compliance@qub.ac.uk.[21]

Part V: Cross-Border Verification & The Common Travel Area

A significant logistical hurdle is verifying identity from London without appearing in person.

The Common Travel Area (CTA) Relevance

The CTA arrangement between the UK and Ireland ensures reciprocal rights, but data controllers are risk-averse regarding identity theft.[22]

- **Verification Standard:** Public bodies in NI and ROI typically require a "Certified Copy" of ID for remote requests.
- **Action:** Do not send original documents.
    - Take your **Irish Passport** or **Passport Card** to a solicitor or Notary Public in London.
    - Have them certify it as a "True copy of the original and a true likeness of the holder."
    - Scan this certified document. This high-trust document usually satisfies the ID requirement for remote SARs.

Part VI: Master Templates for Execution

These templates are engineered to be modular. They integrate the legal arguments for digital access and the "Serious Harm" safety valve.

Template 1: University of Galway (Comprehensive SAR)

To: Data Protection Officer (dataprotection@nuigalway.ie)
Cc: Dean of Students Office (deanofstudents@universityofgalway.ie)
Subject: URGENT: Subject Access Request (GDPR) – – Request for Digital Delivery as Reasonable Accommodation
Legal Reference: Article 15 GDPR; Equal Status Act 2000 (Reasonable Accommodation)
Dear Data Protection Officer,
I am writing to formally make a Subject Access Request for all personal data held by the University of Galway concerning me.
1. Identity & Verification
I am an Irish national currently resident in London. Please find attached a certified copy of my Passport for verification of my identity.
2. Scope of Request
I require all data held in the following departments from to:

- **Student Counselling Service:** All clinical notes, referral letters, intake forms, and case management logs.
- **Student Health Unit:** All medical notes, prescriptions, and correspondence.
- **Office of the President & Office of the Dean of Students:** All emails, memos, welfare reports, and hard-copy files referencing my name, student number, or case. *Please perform a keyword search for on the email archives of these offices.*
- **Security Office:** All incident logs, reports, and any CCTV footage preserved as part of an investigation or disciplinary file.

3. Specific Format Requirement (Reasonable Accommodation)
I live with **Traumatic Brain Injury (TBI)** and **Complex PTSD (C-PTSD)**. These conditions significantly impair my executive function, working memory, and ability to process large volumes of physical text.

- **Strict Digital Requirement:** Under **Section 4 of the Equal Status Act 2000**, I request a **Reasonable Accommodation** that all data be delivered in **digital format** (Searchable PDF with OCR for documents; standard digital video formats for CCTV).
- Providing paper records would render the data functionally inaccessible to me, constituting a discriminatory failure to accommodate my disability.
- Please confirm receipt of this accommodation request specifically.

4. Mental Health Data & Nominated Professional (S.I. No. 121 of 2022)
I am aware that under **Section 60 of the Data Protection Act 2018** and **S.I. No. 121 of 2022**, you may restrict access to health data if you believe it would cause serious harm.

- To avoid delay or refusal, I hereby **nominate the following health professional** to

receive any such restricted data on my behalf:
    - **Name:**
    - **Role:** [GP / Consultant Psychiatrist]
    - **Address:** [Practice Address, London]
    - **Email:** [Email]

I look forward to your response within the statutory one-month period.
Yours sincerely,

Template 2: HSE West / Saolta Group (UCHG Records)

To: Freedom of Information / Consumer Affairs (foi.guh@hse.ie)
Subject: Subject Access Request – University College Hospital Galway – – NIMIS & Mental Health Records

1. Scope:

- **Mental Health Unit:** All inpatient and outpatient notes, multidisciplinary team meeting logs, nursing notes, and risk assessments held by the Department of Psychiatry UCHG.
- **Radiology (NIMIS):** A full digital copy of all MRI, X-ray, and CT imaging held on the **NIMIS** system (DICOM format required) along with the associated Radiologist reports.

2. Reasonable Accommodation (Equal Status Act 2000):
As a resident of London with a disability (TBI/C-PTSD) that prevents effective processing of paper records:

- I require all textual records to be **digitized (scanned to PDF)** and transferred electronically via secure file transfer (e.g., Healthlink or secure email).
- For NIMIS Imaging: I request a secure electronic transfer. If this is not technically feasible, please dispatch the data on CD/DVD to my London address via tracked post. I cannot collect in person.

3. Nominated Health Professional:
If any mental health notes are deemed likely to cause serious harm (Section 60 DPA 2018), do not withhold them. Release them immediately to my nominated GP:.

Template 3: Belfast Health & Social Care Trust (Psychiatric & RVH)

To: Data Protection Officer (DataProtection@belfasttrust.hscni.net)
Subject: Subject Access Request (UK GDPR) – – Ward 5 Psychiatric Notes & RVH Records
Legal Reference: UK GDPR; Disability Discrimination Act 1995 (NI)

1. Scope:

- **Belfast City Hospital (Ward 5):** Full clinical notes from **Psychiatric Ward 5**, including admission/discharge summaries, nursing notes, observation logs, and risk assessments.
- **Royal Victoria Hospital:** All A&E attendance records (Symphony system exports), MRI/X-ray reports (and digital images from **NIPACS**), and consultant correspondence.

2. Whistleblowing Cross-Reference:

- Please search for all internal emails regarding my care or admission that reference

"Alistair Finlay," "Queen's University," or "Whistleblowing," as these communications are vital to my personal context.

3. Serious Harm Test (Schedule 3, DPA 2018):
I assert that accessing these notes is part of my recovery process. However, should you deem any portion exempt under the 'Serious Harm' test, I request you exercise your discretion to release those specific portions to my nominated London psychiatrist/GP:.

4. Reasonable Adjustment (DDA 1995):
Due to TBI and C-PTSD, I require **digital delivery**. Paper records put me at a substantial disadvantage due to cognitive impairment. I request secure electronic transfer as a **Reasonable Adjustment** under the **Disability Discrimination Act 1995**.

Template 4: Queen's University Belfast (Hybrid SAR/FOI)

To: Information Compliance Unit (info.compliance@qub.ac.uk)
Subject: Hybrid Data Request (SAR & FOI) – – Whistleblowing & Alistair Finlay Governance
Part A: Subject Access Request (Personal Data)
Under the UK GDPR, I request:

1. **Alistair Finlay Correspondence:** All emails sent/received by Alistair Finlay (Acting University Secretary) that contain my personal data (Name, Student ID) or refer to my case.
2. **Wellbeing & Security:** All case notes held by Student Wellbeing and Security Services regarding my interactions with the University.

Part B: Freedom of Information Request (Governance Data)
Under the Freedom of Information Act 2000, I request:

1. **Whistleblowing Policy:** The specific QUB Whistleblowing Policy document in force during.
2. **Drafting Notes:** Any meeting minutes, memos, or governance documents authored or approved by Alistair Finlay regarding the drafting, implementation, or review of the University's Whistleblowing Policy during his tenure.

Reasonable Adjustment:
As per the DDA 1995, please provide all responses in digital format (PDF) to accommodate my disability (TBI).

Part VII: Execution Strategy & Troubleshooting

1. Handling "Excessive Request" Refusals

Institutions may claim your request is "manifestly unfounded or excessive" due to the volume of emails requested (especially QUB/Galway).

- **Rebuttal:** "My request is not excessive given the complexity of my medical history and the specific legal context (whistleblowing). The volume of data is a reflection of the

University's extensive processing of my data, not of my request. I have provided specific keywords to assist the search."

2. If They Refuse Digital Formats (Security Argument)

- **Rebuttal:** "GDPR requires you to facilitate rights. As a public body, you possess secure infrastructure (e.g., Cryptshare, SharePoint). Refusal to use this infrastructure to accommodate a disabled data subject is a failure of Reasonable Accommodation/Adjustment under the."

3. Escalation Path

If the deadline (1 month) passes or access is refused:

- **Ireland:** Complaint to the **Data Protection Commission (DPC)**. Specifically cite the failure to provide Reasonable Accommodation under S.I. 121/2022.
- **UK (NI):** Complaint to the **Information Commissioner's Office (ICO) Northern Ireland**. Cite the DDA 1995 breach.

By systematically applying this framework, utilizing the specific templates, and proactively nominating a health professional, the barriers to access—geographic, physical, and legal—can be effectively dismantled.

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